# The Cloister

*Care worth staying home for.*

Financial projection and DCF valuation · a five-year model

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01 / THE MODEL AND ITS DRIVERS

## What the projection assumes.

The Cloister is modelled as one consolidated institution of two businesses, the address and the family-practice OpCo, with the intra-company OpCo tenancy eliminated. The address opens and ramps in Year 1; the membership launches alongside it. Every driver below can be tuned, and the valuation moves with them.

| Driver | Assumption | Note |
| --- | --- | --- |
| The address, external revenue at run-rate | NGN 339M | Specialist membership, sessions and BQ office leases |
| Membership blended fee | NGN 2.38M / member / yr | Weighted across Individual, Family and Executive tiers |
| Membership ramp (members) | 180 to 680 | Year 1 launch to Year 5, panels of ~350 per physician |
| OpCo direct cost of care | 50% of OpCo revenue | Physicians, nurses, annual screens, admin and marketing |
| Operator fee to C4A | 20% of institution EBITDA | C4A builds and runs both businesses |
| Tax and maintenance | ~30% effective | Nigerian company tax and maintenance capex |
| Discount rate (NGN nominal) | 28% | Nigerian cost of capital |
| Terminal growth (NGN nominal) | 12% | Long-run nominal, below inflation-plus-real growth |

Figures are in NGN nominal, priced to Lagos cash-pay demand and benchmarked to premium London and Lagos practice (concept note, sections 05 and 06). The naira is inflationary, so the 28% discount rate and 12% terminal growth are stated together and should be tuned as a pair.

02 / REVENUE

## Two engines, ramping. NGN millions.

| Revenue (NGN M) | Year 1 | Year 2 | Year 3 | Year 4 | Year 5 |
|---|---|---|---|---|---|
| The address (membership, sessions, offices) | 186 | 339 | 380 | 407 | 427 |
| Membership members (count) | 180 | 380 | 500 | 600 | 680 |
| The membership OpCo (revenue) | 428 | 905 | 1,190 | 1,428 | 1,618 |
| **Total institution revenue** | **614** | **1,244** | **1,570** | **1,835** | **2,045** |

The address reaches its run-rate by Year 2 and grows modestly on sell-through and rate. The membership is the growth engine: it launches at about 180 members, hits its Year-2 target of 380, and builds toward 680 as the panels fill. By Year 5 the institution is turning over more than NGN 2 billion a year, the great majority of it recurring.

03 / PROFITABILITY

# From revenue to Medbury's earnings. NGN millions.

| Profit (NGN M) | Year 1 | Year 2 | Year 3 | Year 4 | Year 5 |
|---|---|---|---|---|---|
| Total institution revenue | 614 | 1,244 | 1,570 | 1,835 | 2,045 |
| Less: building hold cost | (92) | (97) | (102) | (107) | (112) |
| Less: OpCo cost of care (50%) | (214) | (453) | (595) | (714) | (809) |
| **Institution EBITDA** | **308** | **694** | **873** | **1,014** | **1,124** |
| Less: operator fee to C4A (20%) | (62) | (139) | (175) | (203) | (225) |
| **Medbury EBITDA** | **246** | **555** | **698** | **811** | **899** |

The address covers its own building cost from a light base; the margin comes from the sessions and, above all, the membership, which carries a healthy contribution once the panels are full. After C4A's operator fee, Medbury's own EBITDA runs from about NGN 246M in the launch year to nearly NGN 900M by Year 5, on an upfront investment of roughly NGN 150M across the address fit-out and the OpCo launch.

## The clinical headcount is lean.

| Staffing and panels | Year 1 | Year 2 | Year 3 | Year 4 | Year 5 |
|---|---|---|---|---|---|
| Members | 180 | 380 | 500 | 600 | 680 |
| **Family physicians (employed)** | **2** | **2** | **2** | **3** | **3** |
| Panel per physician (members) | 90 | 190 | 250 | 200 | 227 |
| Nurses and coordinators | 3 | 4 | 4 | 5 | 5 |
| Specialists on roster (affiliated) | 15 | 25 | 30 | 35 | 40 |

The institution employs only two to three family physicians, on premium panels held deliberately small, plus a handful of nurses and coordinators. The specialists are a paid-us roster of affiliated members, not payroll, which is the whole margin lever. On space, the family practice holds two to three of the seven rooms; 500 members is the point at which a second address is triggered, not the building overcrowded.

04 / VALUATION

## Discounted cash flow. NGN millions.

| Discounted cash flow (NGN M) | Year 1 | Year 2 | Year 3 | Year 4 | Year 5 |
|---|---|---|---|---|---|
| Medbury EBITDA | 246 | 555 | 698 | 811 | 899 |
| **Free cash flow (after tax, maintenance)** | **172** | **389** | **489** | **568** | **629** |
| Discount factor at 28% | 0.78 | 0.61 | 0.48 | 0.37 | 0.29 |
| Present value of free cash flow | 134 | 237 | 233 | 212 | 183 |

| Valuation build | Amount |
|---|---|
| Sum of PV, Years 1 to 5 free cash flow | NGN 999M |
| Terminal value at end of Year 5 (12% growth) | NGN 4,403M |
| Present value of terminal value | NGN 1,281M |
| **Enterprise value of The Cloister** | **NGN 2,280M** |

On these assumptions The Cloister is worth about **NGN 2.3 billion**, against an upfront investment of roughly NGN 150M. The institution pays that upfront back inside the first year of run-rate and compounds from there, and the valuation still excludes the diagnostics, pharmacy and procedure revenue the two engines drive into the wider Medbury network.

05 / SCENARIO ANALYSIS

## Three ways it could go.

The base case is deliberately conservative. The value turns most on how fast the membership fills and how hard the rooms are used, so the model is run in three scenarios that flex those operational drivers together. The discount rate is held at 28% across all three; the grid overleaf varies it separately.

| Scenario drivers | Bear | Base | Bull |
|---|---|---|---|
| Members by Year 5 | 500 | 680 | 1,050 |
| Consulting sell-through | ~45% | ~55% | ~65% |
| OpCo cost of care | 52% | 50% | 48% |
| Membership ramp | Slow | Steady | Fast |

| Outcome | Bear | Base | Bull |
|---|---|---|---|
| Year-5 total revenue (NGN Bn) | 1.5 | 2.0 | 3.0 |
| Year-5 Medbury EBITDA (NGN M) | 611 | 899 | 1,366 |
| **Enterprise value (NGN Bn)** | **1.5** | **2.3** | **3.4** |

Even the bear case, a slow membership build with rooms half empty, values the institution at about NGN 1.5 billion, some ten times the upfront. The bull case, a fast fill toward a thousand members, reaches NGN 3.4 billion. The downside is protected by the recurring memberships, offices and OpCo tenancy, which cover the building before a room is used.

06 / SENSITIVITY

## What moves the number.

The valuation is most sensitive to the discount rate and the terminal growth, the pair that captures the naira's inflation and Nigeria's cost of capital, and to the membership ramp. The grid shows the enterprise value, in NGN billions, across a reasonable range of the first two.

| Enterprise value (NGN Bn) | Growth 10% | Growth 12% | Growth 14% |
|---|---|---|---|
| Discount 25% | 2.5 | 2.8 | 3.3 |
| **Discount 28% (base)** | **2.1** | **2.3** | **2.6** |
| Discount 32% | 1.6 | 1.7 | 1.9 |

Across the range the institution is worth between roughly NGN 1.6 billion and NGN 3.3 billion, with a central case near NGN 2.3 billion. The membership ramp is the other lever: reaching the Year-2 target of 380 members a year earlier, or building past 680, lifts every figure above.

### A note on the assumptions.

This is an illustrative model, not a diligence-grade one. The revenue drivers are benchmarked to premium London and Lagos practice and to the concept note; the cost of care, the operator fee, the discount rate and the terminal growth are stated so they can be set against real figures once a building is shortlisted, the Med Lyfe rates are live, and the OpCo staffing is priced. The intra-company OpCo tenancy is eliminated in consolidation. Figures are NGN nominal.

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